

VOB Section — Review & Improvement Recommendations

Revision 2 — language and terminology for the Provider and Office Manager views

Item	Detail
Screen reviewed	Revusys / VOB Admin Dashboard — software.framecho.ae/admin/dashboard
Roles in scope	Provider (Doctor) and Office Manager
Focus of this revision	The words on screen — do the labels mean anything to a provider or a front-desk manager, and do they say what to do next
Evidence	3 screen captures of the live environment, dated Aug 25, 2026
Contains	Proposed wording for every label in the section, two visual mockups, 18 screen findings, and 6 questions for the VOB team

What changed in Revision 2

Revision 1 treated the wording as a polish item. This revision leads with it. The eligibility statuses are re-cut into two separate questions (is the policy active, and are we in network); the five processing statuses are re-written for both roles and grouped by who is holding the work; Top Agents and Recent Activity Logs are removed from both the Provider and the Office Manager views; and every proposed label is tied to the action it should trigger.

1. Executive summary

The VOB dashboard is well built, but it is written in the language of the VOB team rather than the language of the people reading it. Every label on the screen names an internal workflow state — Pending QC, Returned, Drafts, Ineligible — and none of them tells a provider or a front-desk manager what to do next.

Four points carry the rest of this document:

- “Ineligible” is doing too much work. It currently covers a terminated policy, a service that is not a covered benefit, a member the insurer cannot find, and — most importantly — a patient we are out of network with. Those are four different phone calls, shown as one number.
- “Approved” is the riskiest word on the screen. In a clinical setting it reads as the insurer approving treatment. It actually means our own team approved the record. On a provider-facing screen this should never say “Approved”.
- The five processing statuses are internal handoffs. A provider needs three buckets: ready to treat, we are working on it, or we need something from your office.
- Top Agents and Recent Activity Logs do not belong in either role's view. Staff-level productivity and system audit trails are administrative content and should be admin-only.

Section 9 is a single reference table covering every label in the section, with a column for the Provider view and a column for the Office Manager view. Section 10 lists six questions that need answering by the VOB team before some of these labels can be finalised.

2. The principle behind every change

One rule explains almost every recommendation in this document:

A label should name the situation the patient is in, and imply who has to act next — not the queue the record is sitting in.

“Pending QC” names a queue. “Being checked by our team” names a situation and tells the reader there is nothing for them to do. Same record, same data, different reader experience. Applying that single rule to the VOB section removes most of the confusion described below.

Three supporting rules:

- Say what the front desk should do about it. If a status does not imply an action, it needs a sub-line that does.
- Never use an internal abbreviation on a screen a provider sees — QC, VOB, OON — without expanding it at least once.
- Do not use a word that has a different, stronger meaning in a clinical context. “Approved”, “Denied” and “Authorised” all mean something specific to a provider.

3. Eligibility language — the most important change

Today the section shows three eligibility figures, and a donut chart of the same three with no labels at all.

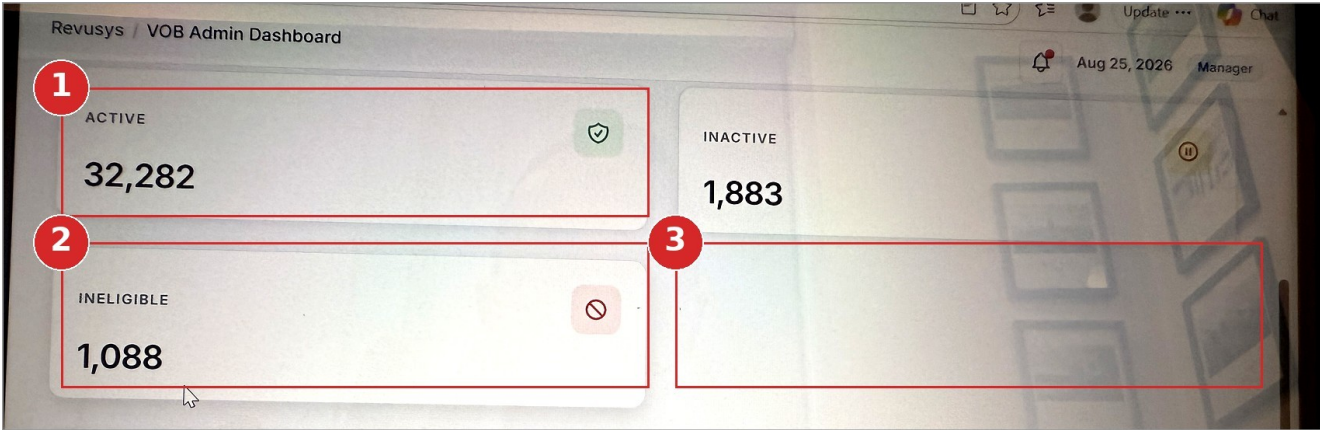


Figure 1 — Today: Active, Inactive and Ineligible, with no definitions and no drill-down

3.1 What is wrong with the current three words

Current label	The problem	What a reader assumes
Active	Does not say active what — the patient, the patient's record, or the insurance policy.	A Doctor reads “active patient”. It actually means active coverage.
Inactive	Reads as an inactive patient record — someone who has left the practice.	Nobody follows up, because it sounds like a housekeeping status rather than a coverage problem.
Ineligible	Covers at least four different situations, each needing a different action. It also sounds permanent and sounds like the patient is at fault.	“This patient cannot be treated” — which is often not true, particularly for an out-of-network patient.

The out-of-network point

A patient we are out of network with usually has perfectly valid, active insurance. Filing them under “Ineligible” is inaccurate and it hides the one case that most needs a conversation before treatment — the patient's financial responsibility. Out of network is not an eligibility outcome; it is a separate question about our relationship with the payer, and it deserves its own field.

3.2 The recommendation — split one question into two

Eligibility is currently answering two different questions with one word. Separating them makes both answerable at a glance.

Insurance Coverage

Two separate questions: is the policy active, and are we in network? Today both are collapsed into one word — "Ineligible".

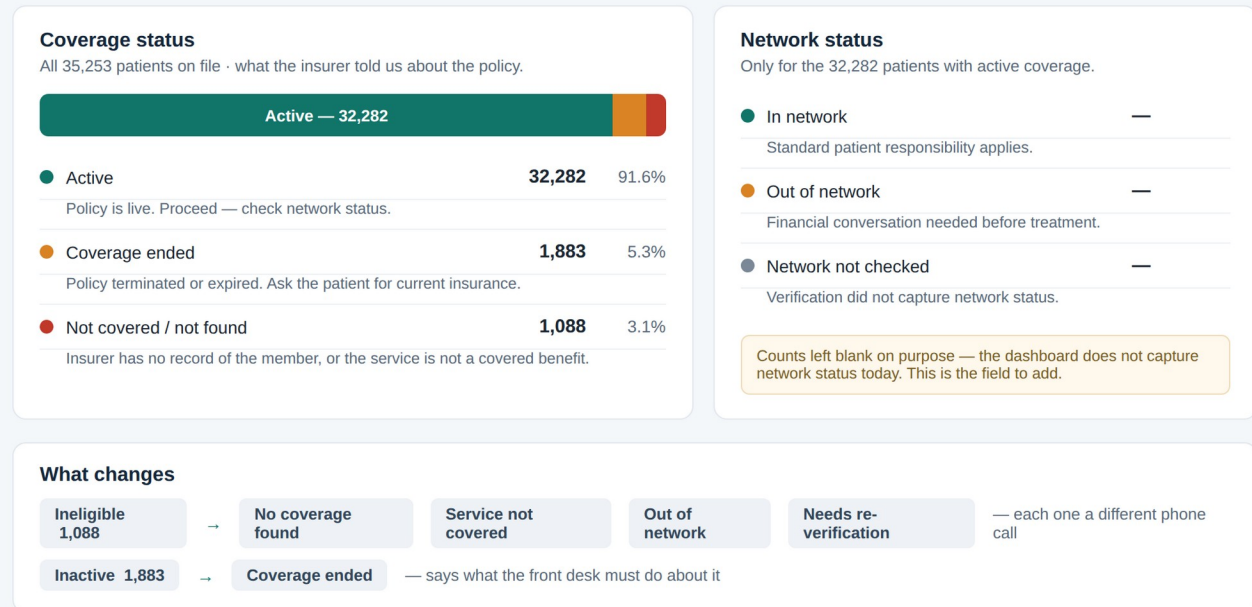


Figure 2 — Proposed: coverage status and network status shown as two separate questions

Question 1 — Is the policy active? (all patients)

Proposed label	Replaces	What the front desk does
Active	Active	Nothing — proceed, then check network status.
Coverage ended	Inactive, and part of Ineligible	Ask the patient for their current insurance card.
Not covered	Part of Ineligible	The plan is active but does not cover this service. Discuss self-pay or an alternative.
No coverage found	Part of Ineligible	The insurer has no record of this member. Re-check the member ID, date of birth and payer.
Needs re-verification	Not currently shown	Our own check is out of date. Re-run before treating.

Question 2 — Are we in network? (patients with active coverage only)

Proposed label	What the front desk does
In network	Standard patient responsibility applies. Proceed.
Out of network	Have the financial conversation before treatment — higher out-of-pocket, possible out-of-network benefits, or a waiver.

Proposed label	What the front desk does
Network not checked	The verification did not capture network status. Confirm before scheduling.

Two terms that are being confused today — worth separating explicitly

“Expired” can mean two completely different things, and they need different words. A policy that has ended is “Coverage ended” — the patient must supply new insurance. A verification we ran 90 days ago that is now stale is “Needs re-verification” — the patient's insurance may be perfectly fine, but our information is old. Using one word for both means one of the two never gets actioned.

4. Processing status language

The processing chart is the clearest component on the screen — but the five statuses in it are internal handoffs between members of the VOB team, not states a provider or an office manager would recognise.

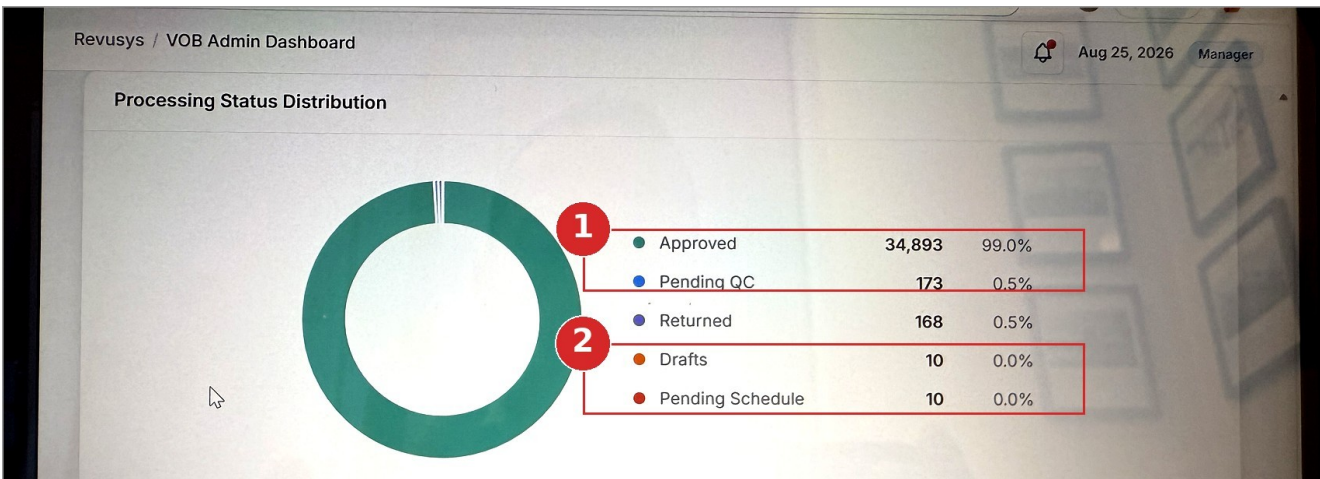


Figure 3 — Today: Approved, Pending QC, Returned, Drafts and Pending Schedule

4.1 What each status is really saying

Current label	The problem	Risk if left as is
Approved	Reads as the insurer approving treatment or authorising a procedure. It means our internal quality check passed.	A provider treats on the assumption that the payer has authorised the service. This is the highest-risk word on the screen.
Pending QC	“QC” is an internal department function. It does not say who is waiting on whom.	Front desk chases a queue that is already moving, or assumes the practice must act.
Returned	Returned by whom, to whom? It reads as the insurer returning or rejecting the claim.	Reads as a denial when it is an internal correction.
Drafts	Whose draft, and is anyone working on it?	Sits untouched because no owner is implied.
Pending Schedule	Ambiguous — is the verification waiting on the patient to book, or blocked until an appointment exists?	Neither side acts, because neither knows it is theirs. See question 1 in section 10.

4.2 For the Provider — three buckets, not five

A provider only needs to know whether they can treat this patient, and if not, who is holding it up. The same five statuses roll up cleanly into three.

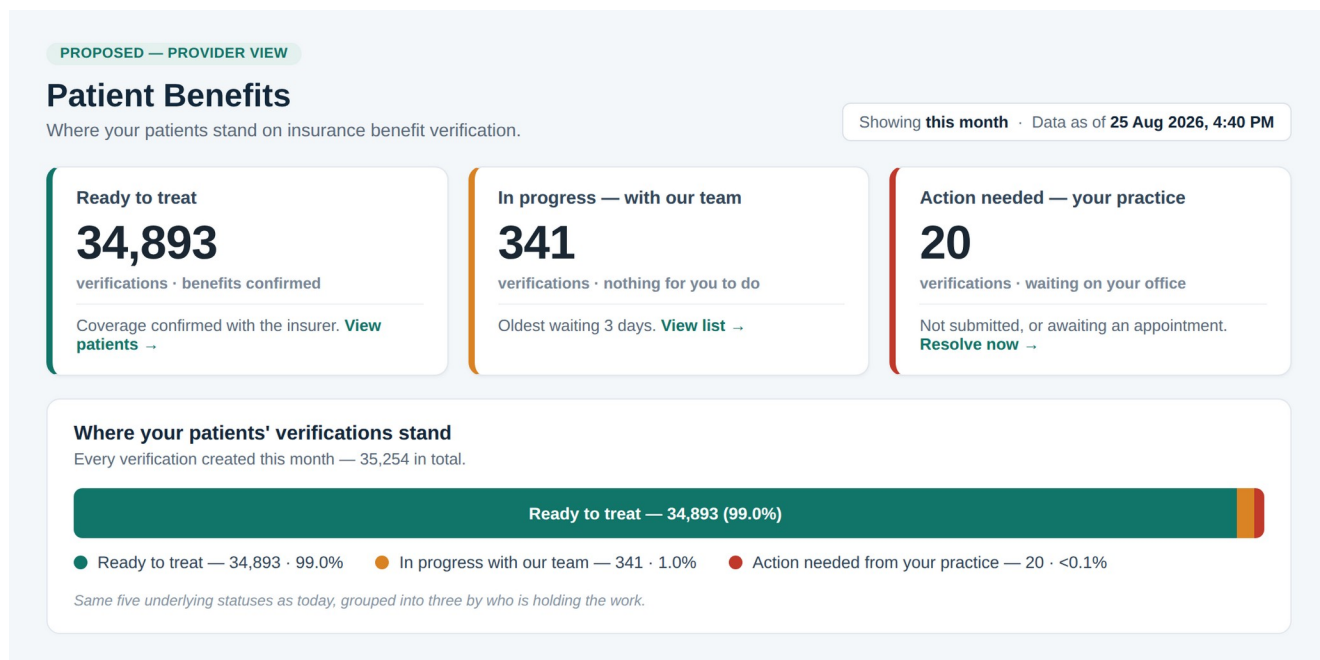


Figure 4 — Proposed Provider view: the same data grouped by who is holding the work

Provider sees	Rolls up	Why this grouping
Ready to treat	Approved (34,893)	Benefits are confirmed. The only bucket that means “go”.
In progress — with our team	Pending QC + Returned (341)	Our team is still working. Nothing for the practice to do — which is exactly what a provider needs to know.
Action needed — your practice	Drafts + Pending Schedule (20)	Something is waiting on the office. This is the only bucket that should generate a chase.

4.3 For the Office Manager — keep five, rename all five

The Office Manager does need the detail, because they are the person chasing each queue. The statuses stay, the words change, and they are grouped by who is holding the work.

Current	Office Manager sees	Who holds it	Count
Approved	Benefits confirmed	Done	34,893
Pending QC	In quality review	Our team	173
Returned	Returned for correction	Our team	168
Drafts	Started — not submitted	The practice	10
Pending Schedule	Waiting on appointment	The practice	10

One word to avoid entirely on provider-facing screens

Do not use “Approved”, even shortened or in a tooltip, anywhere a provider can see it. “Benefits confirmed” carries the same meaning without colliding with prior authorisation. Reserve “Approved” for the internal admin view, where the audience knows it refers to the QC step.

5. Metric card language

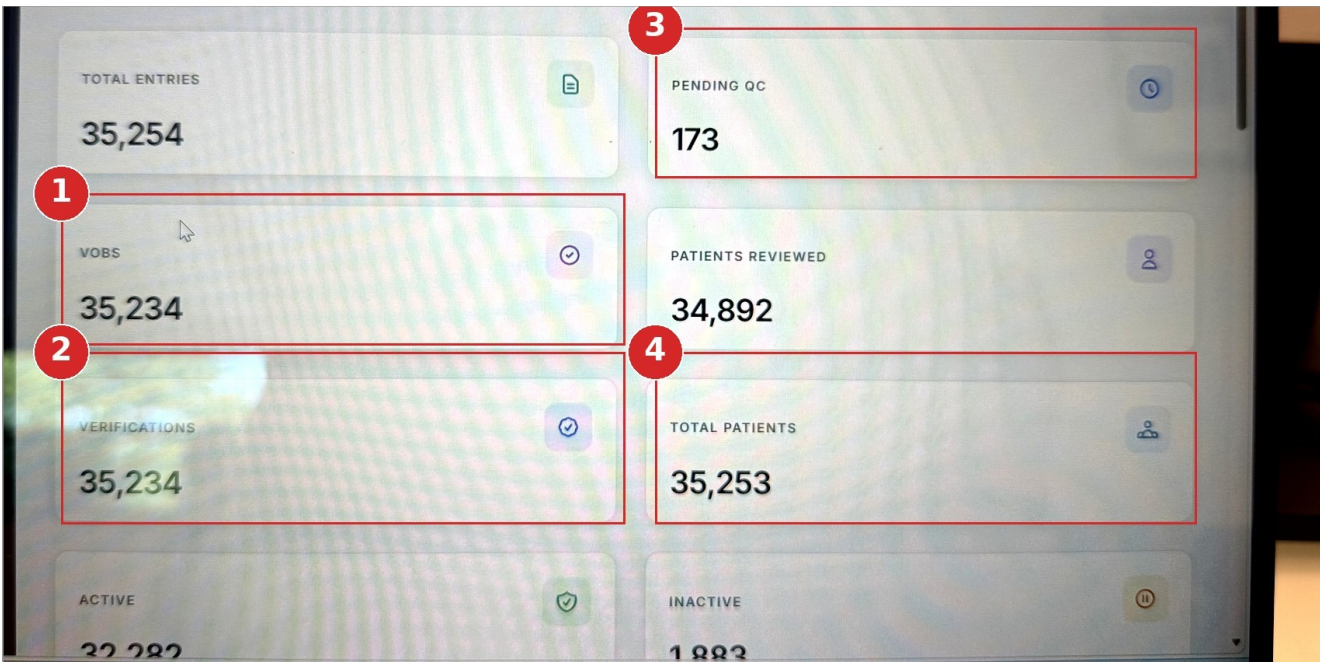


Figure 5 — Today: eight cards, all in internal system language

“Total Entries” is the clearest example of the problem. An entry is a database row. A provider does not think in entries, and an office manager does not chase entries — they chase patients and verifications. Every card should name the thing being counted, in the reader's words.

Current card	Provider sees	Office Manager sees	Notes
TOTAL ENTRIES	Not shown	Verifications created	A row count means nothing to a provider
VOBS	Not shown	Verifications completed	Duplicate of the card below — keep one
VERIFICATIONS	Benefits confirmed	Verifications completed	Both cards show 35,234 today — see F4
PENDING QC	In progress — our team	In quality review	Provider version says there is nothing to do
PATIENTS REVIEWED	Patients ready to treat	Patients with confirmed benefits	“Reviewed” does not say by whom or with what result
TOTAL PATIENTS	My patients	Patients on file	Provider version scoped to their own panel
ACTIVE	Active coverage	Active coverage	Says the status belongs to the policy

Current card	Provider sees	Office Manager sees	Notes
INACTIVE	Coverage ended	Coverage ended	Implies the action — get a new card
INELIGIBLE	Replaced — see §3	Replaced — see §3	Split into four outcomes plus network status

Two supporting changes that make the cards readable:

- Always state the unit under the number — “34,893 verifications” reads differently from “34,893 patients”, and the current screen mixes the two without saying so.
- Drop the ALL CAPS labels. Sentence case is faster to read and lets the important cards stand out.

6. What to remove from both views

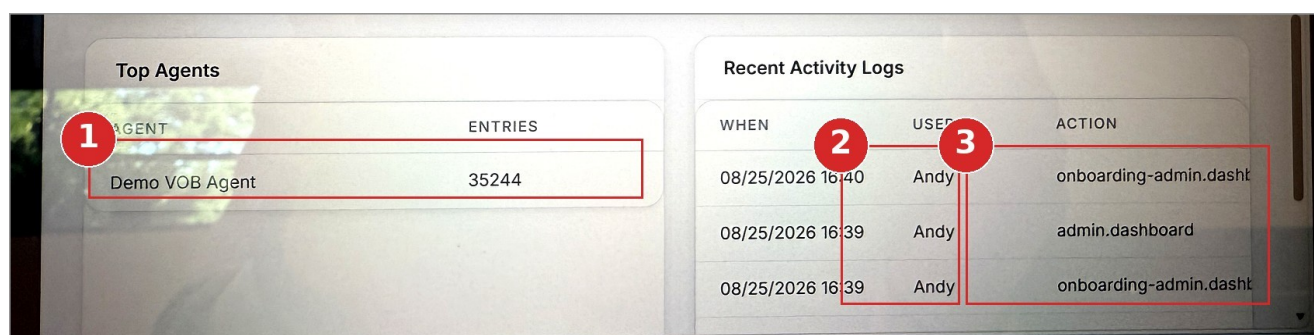


Figure 6 — Top Agents and Recent Activity Logs: administrative content in a clinical view

Panel	Why it should be admin-only
Top Agents	Named staff productivity is a staffing and performance matter. It informs no clinical or front-desk decision, and showing individual agents' output to an external provider is not appropriate. It currently shows a single demo record, which makes the point moot but also damages credibility.
Recent Activity Logs	An audit trail exists for compliance and troubleshooting. It shows raw system route names, identifies users by first name only, and answers a question neither role is asking. Keep it, move it to the admin area, and rewrite the actions in plain English there.

Removing both panels frees the entire lower half of the screen. That space is better spent on the two things neither role can currently reach: the list of patients needing action, and the out-of-network patients who need a financial conversation before treatment.

7. What each role should see

7.1 Provider (Doctor)

Show	Remove
• Ready to treat — benefits confirmed • In progress — with our team • Action needed — your practice • Out of network — financial conversation needed • Coverage ending in the next 30 days • Patient drill-down: carrier, plan, effective dates, copay, deductible met, out-of-pocket remaining, authorisation required	• Top Agents • Recent Activity Logs • Total Entries • Pending QC as a standalone card • Operational Processing Overview • Any use of the words QC, entry, draft or approved

7.2 Office Manager

The current layout is close to right for this role. The change needed is that it reports where it should direct — every number should be a way into the work.

- Every status is a link into its filtered queue: In quality review, Returned for correction, Started — not submitted, Waiting on appointment, and each coverage outcome.
- Group the queues by who is holding the work — our team, or the practice — so it is obvious what can actually be chased.
- Add ageing to every queue: oldest item, and how many are over target.
- Add an out-of-network worklist — these patients need a financial conversation before their appointment, and today there is no way to find them.
- Keep the Day-End Report, and let it export the current period directly to PDF or Excel.

8. Remaining screen findings

These are the non-language issues from the first review, still outstanding. Priorities: P1 affects accuracy or trust, P2 is a usability gap, P3 is presentation polish.

ID	Finding	Priority	Where
F1	Header date is unlabelled and does not match the current date; no period filter, refresh or “last updated” stamp	P1	Header
F2	Page title says “Admin Dashboard” to users who are not admins; VOB is never expanded	P2	Header
F3	Four tabs have no selected state; “Day End Report” vs sidebar “Day-End Report”	P2	Header
F4	“VOBs” and “Verifications” show the identical value (35,234) with no stated difference	P1	Cards

ID	Finding	Priority	Where
F5	No definitions or tooltips on any card; units are never stated	P2	Cards
F6	No card or status is clickable — the work cannot be reached from the numbers	P2	Whole page
F7	ALL CAPS labels; no trend or comparison against a previous period	P3	Cards
F8	Eligibility figures sum exactly to Total Patients, but the relationship is never shown	P2	Cards
F9	“Ineligible” has no drill-down and no reason breakdown	P2	Cards
F10	Ineligible card sits alone leaving a large empty panel, which reads as a broken card	P3	Layout
F11	Eligibility donut has no legend, labels, values or tooltips; colour is the only encoding	P2	Charts
F12	Operational Processing Overview compares four near-identical values; Pending QC is invisible	P2	Charts
F13	Approved 34,893 does not reconcile with Patients Reviewed 34,892; no denominator stated	P1	Charts
F14	Non-zero counts of 10 are displayed as “0.0%”, which reads as nothing to do	P1	Charts
F15	Demo data (“Demo VOB Agent”) is visible; 35244 is missing its thousands separator	P1	Tables
F16	Activity log shows raw system route names, truncated mid-word	P2	Tables
F17	Users identified by first name only; date format differs from the rest of the page	P3	Tables
F18	Both roles are shown the same admin screen	P2	Roles

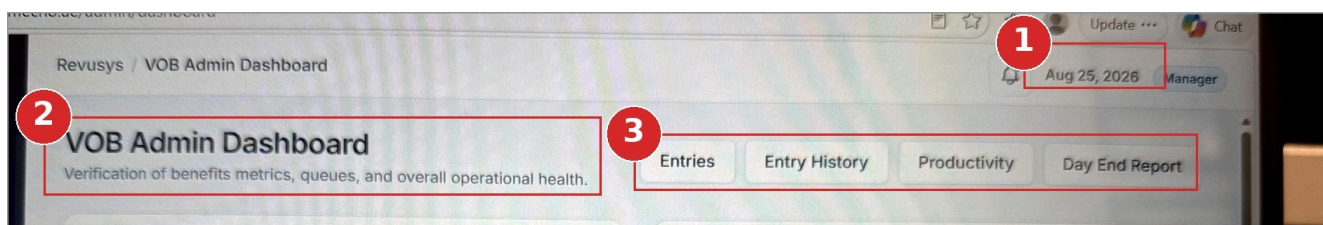


Figure 7 — Header: unlabelled date (F1), admin-facing title (F2), tabs with no selected state (F3)

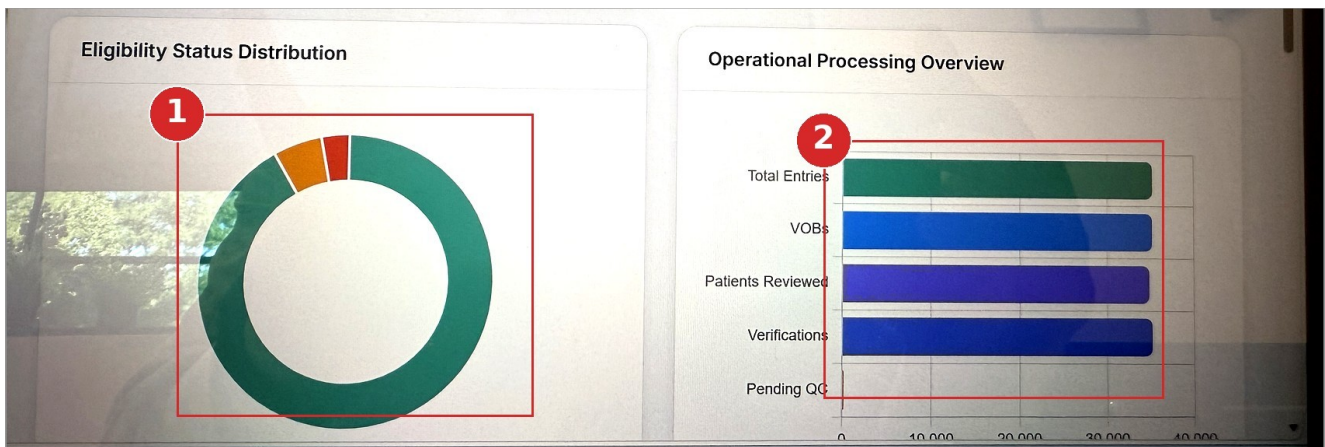


Figure 8 — Charts: unlabelled donut (F11) and four near-identical bars (F12)

9. Full wording reference

Every label in the VOB section, with the proposed wording for each role. Where a cell says “Not shown”, that item should not appear in that role's view at all.

Current wording	Provider view	Office Manager view	Why
VOB Admin Dashboard	Patient Benefits	Benefits Verification	Neither role is an admin
Verification of benefits metrics, queues, and overall operational health.	Where your patients stand on insurance benefit verification.	Track verifications, quality review and patient coverage.	Describes the reader's job, not the system
Total Entries	Not shown	Verifications created	“Entry” is a database row
VOBs / Verifications	Benefits confirmed	Verifications completed	One card, not two
Pending QC	In progress — with our team	In quality review	“QC” is internal; provider version says there is nothing to do
Returned	In progress — with our team	Returned for correction	Avoids reading as an insurer denial
Drafts	Action needed — your practice	Started — not submitted	Implies an owner
Pending Schedule	Action needed — your practice	Waiting on appointment	Subject to question 1, §10
Approved	Ready to treat	Benefits confirmed	Never “Approved” on a provider screen
Patients Reviewed	Patients ready to treat	Patients with confirmed benefits	States by whom and with what result
Total Patients	My patients	Patients on file	Provider version scoped to their panel
Active	Active coverage	Active coverage	The policy is active, not the patient
Inactive	Coverage ended	Coverage ended	Implies the action required
Ineligible	Split — see below	Split — see below	Four outcomes, not one
— (part of Ineligible)	Not covered	Service not covered	Plan is active, service is not a benefit

Current wording	Provider view	Office Manager view	Why
— (part of Ineligible)	No coverage found	No coverage found	Insurer has no record of the member
— (part of Ineligible)	Out of network	Out of network	Not an eligibility outcome — its own field
— (not shown today)	Needs re-verification	Needs re-verification	Our check is stale, not the policy
Top Agents	Not shown	Not shown	Admin only — §6
Recent Activity Logs	Not shown	Not shown	Admin only — §6
Day End Report / Day-End Report	Not shown	Day-End Report	Currently inconsistent between tab and sidebar
Entries / Entry History	Not shown	Today's Verifications / All Verifications	Makes the two views distinguishable
Aug 25, 2026 (unlabelled)	Data as of 25 Aug 2026, 4:40 PM	Data as of 25 Aug 2026, 4:40 PM	Removes the ambiguity
0.0% (for a count of 10)	<0.1%	<0.1%	Never show a real backlog as zero

10. Questions for the VOB team

Six of the recommendations above depend on how the system actually behaves. These need confirming before the wording is finalised.

1. Does “Pending Schedule” mean the verification is complete and waiting for the patient to book, or that it cannot proceed until an appointment exists? The two need opposite wording — “Ready — awaiting booking” against “Blocked — needs an appointment”.
2. Does “Approved” mean our internal quality check passed, or that the payer approved or authorised the service? The whole of section 4 assumes the former.
3. Is “Ineligible” a single response the payer returns, or a bucket the system assigns when several different checks fail? If it is a bucket, the underlying reasons already exist and only need surfacing.
4. Is network status captured during verification today, in any field? If it is, the out-of-network split in section 3 can be built now. If not, it is a new field on the verification form.
5. Do “VOBs” and “Verifications” have genuinely different definitions? They show the same number today, and both equal Total Entries minus Drafts and Pending Schedule.
6. Is there a re-verification policy — after 30, 60 or 90 days? “Needs re-verification” only works if there is an agreed shelf life for a completed check.

11. Suggested order of work

Stage 1 — wording only, no data model changes

- Rename every label per section 9 for the Office Manager view.
- Replace “Approved” with “Benefits confirmed” everywhere a provider can see it.
- Rename “Inactive” to “Coverage ended” and add the action sub-line to each status.
- Fix the “0.0%” display, the missing thousands separator, and the Day-End Report inconsistency.
- Label the header date, and remove the demo agent record.

Stage 2 — grouping, links and the eligibility split

- Split “Ineligible” into its underlying outcomes (subject to question 3).
- Add network status as its own field and worklist (subject to question 4).
- Group the five processing statuses by who is holding the work, and make each one a link into its queue.
- Add the period selector, tooltips and “as of” stamp.
- Move Top Agents and Recent Activity Logs to the admin area.

Stage 3 — the Provider view

- Build the three-bucket Provider view shown in Figure 4, scoped to the provider's own patients.
- Add the patient drill-down with carrier, plan, effective dates and patient financial responsibility.
- Add the “coverage ending in 30 days” and out-of-network worklists.

Appendix A — How the figures on screen reconcile

The numbers do tie together, which is a good sign. The problem is that none of these relationships are visible to the reader.

Relationship	Result	What it tells us
Active 32,282 + Inactive 1,883 + Ineligible 1,088	= 35,253	These three are patient coverage statuses and add up to Total Patients
Approved 34,893 + Pending QC 173 + Returned 168 + Drafts 10 + Pending Schedule 10	= 35,254	The processing statuses add up to Total Entries — the correct denominator for that chart
Total Entries 35,254 – Drafts 10 – Pending Schedule 10	= 35,234	Equals both “VOBs” and “Verifications” — strong evidence the two cards are one measure shown twice (F4)
Approved 34,893 vs Patients Reviewed 34,892	differs by 1	Two measures that look identical but are not — see question 2
Total Entries 35,254 vs Total Patients 35,253	differs by 1	Suggests one patient with two entries, or a leftover test record
Pending QC 173 + Returned 168	= 341	The “in progress with our team” bucket in the proposed Provider view
Drafts 10 + Pending Schedule 10	= 20	The “action needed from your practice” bucket

Appendix B — Tooltip text, ready to use

Label	Tooltip
Ready to treat	Benefits have been confirmed with the insurer. Nothing further is needed before treatment.
In progress — with our team	Our verification team is still working on these. Nothing is needed from your office.
Action needed — your practice	These are waiting on your office before verification can continue.
Benefits confirmed	The verification is complete and has passed quality review. This does not mean the payer has authorised the service.

Label	Tooltip
In quality review	Completed verifications waiting for a second review before they are released.
Returned for correction	Sent back to the verification agent to fix an error. This is an internal step, not an insurer decision.
Started — not submitted	A verification was begun but never submitted.
Waiting on appointment	Verification cannot be completed until the patient has an appointment booked.
Active coverage	The insurance policy is currently active.
Coverage ended	The policy has been terminated or has expired. Ask the patient for current insurance.
Not covered	The policy is active, but this service is not a covered benefit under the plan.
No coverage found	The insurer has no record of this member. Check the member ID, date of birth and payer.
Out of network	The patient's plan is active, but we are not a participating provider. Discuss patient responsibility before treatment.
Needs re-verification	This verification is older than our re-check policy and should be run again before treatment.

Prepared from screen captures of the live VOB Admin Dashboard dated Aug 25, 2026. Figures 2 and 4 are mockups produced for this document; the counts shown in them are taken from the live figures, except the network status counts, which are deliberately left blank because that field is not captured today. No changes were made to the application as part of this review.